

THE NEW GUIDE TO

LOOKSMAXXING

*Your appearance is your first impression, your billboard,
your silent pitch to the world. Make it count.*

A no-nonsense guide for any man ready to level up.

For informational purposes only. Nothing herein constitutes medical, legal, or financial advice.
Consult qualified professionals before making health-related decisions.

Introduction

Why your appearance is your most underrated asset.

Think of your appearance as the first level of marketing for yourself. It is your billboard, your TV commercial, your business card — all rolled into one. Before you say a single word, the world is already forming an opinion. That opinion is based largely on what it sees.

This guide isn't about vanity. It's about respect — for yourself, and for the people you interact with. A man who invests in how he presents himself signals that he takes himself seriously. And when you take yourself seriously, others follow suit.

You don't need to be born with model genetics. You don't need a huge budget. What you need is awareness and intention. Each section targets a specific area of your appearance, gives you a direct action plan, and in some cases offers optional advanced strategies for those who want to go further. A dedicated peptide reference chapter at the end of this guide covers every compound mentioned in detail, including exact reconstitution math and dosing references based on the most widely cited research protocols.

Key principle: You are not trying to become someone else. You are removing the obstacles between who you are and how you're perceived.

01 — The Body

The foundation everything else is built on.

Among all aspects of your appearance, body composition has the highest return on investment. A reasonably fit physique can compensate for almost any other shortcoming. It signals discipline, health, and vitality — and it does so loudly, even under clothing.

You do not need to look like a Greek marble statue. But the closer you move toward symmetry and proportion, the better. The goal is a body that looks like it belongs to someone who takes care of himself — not a physique built in a lab.

Training: Resistance training is non-negotiable. Whether that's a commercial gym, a home setup, or a bodyweight program, you need to be lifting or loading your muscles consistently. Aim for at least three sessions per week. Focus on compound movements — squats, deadlifts, rows, presses — before adding isolation work. Cardio matters too, both for body composition and for the way you carry yourself.

Nutrition: No training program will outrun a poor diet. Get enough protein (a general rule of thumb is around 0.7–1g per pound of bodyweight), minimize ultra-processed foods, and stay hydrated. Many apps built by legitimate coaches and registered dietitians can take the guesswork out of this entirely.

Advanced options — peptides and compounds: For those who want to go further, several compounds have become significantly more accessible. **Retatrutide**, a triple receptor agonist (GLP-1, GIP, and glucagon), is currently among the most discussed compounds for body composition and fat loss, delivering results that exceed earlier GLP-1 drugs in preliminary trials. **Tirzepatide** (GLP-1/GIP dual agonist) is now obtainable by prescription through several legitimate online men's health clinics. Testosterone replacement therapy can also be assessed and prescribed through these same clinics if bloodwork indicates clinical need. Additionally, **BPC-157** and **TB-500** are widely used in the training community for accelerated injury recovery and joint health — making consistent training more sustainable over time. Full reconstitution and dosing references for all of these are in the Peptide Reference Chapter at the end of this guide.

These are tools — they work alongside training and nutrition, not instead of them. Consult a qualified physician before beginning any hormonal or peptide regimen.

02 — The Clothing

Fit is everything. Everything else is secondary.

Apparel does not need to be expensive to be effective. What it absolutely must be is well-fitted. This single factor — fit — separates men who look put together from men who look like they're borrowing someone else's clothes.

One of the most common mistakes men make is wearing clothing that is too large. The reasoning is usually body insecurity — too thin, too heavy, or simply not comfortable with attention. The irony is that oversized clothing makes all of these things worse, not better. Fitted clothing creates the illusion of a better physique at every size.

Where to start: Visit a tailor and get properly measured. Not to have everything custom-made (although that is the gold standard), but simply to know your actual measurements. Most men have been guessing at their size for years. Once you know your numbers, buying off the rack becomes dramatically more effective.

Building a foundation wardrobe: Trends come and go. Classic, simple styles in neutral colors will always look appropriate and will never date you. Build from that base first.

- At least one tailored suit — ideally three. A dark navy or charcoal, a black, and a lighter neutral such as tan or mid-grey.

- Shoes: a black dress shoe and a brown one cover almost every occasion. Your belt should always match your shoes.
- Casual basics: well-fitted chinos, dark denim, clean white and neutral tees, a quality leather or minimal sneaker.
- One or two quality outerwear pieces — a structured wool coat or a clean bomber will elevate almost any outfit underneath it.

A useful personal rule: dress at least as well as — preferably better than — everyone else in the room you are about to walk into. It becomes a habit that pays dividends consistently.

Budget note: Building this out takes time. Prioritize fit and quality over quantity. One great piece outperforms five mediocre ones every time.

03 — Hair, Grooming & Hair Loss

Cleanliness is the baseline. Keeping what you have is the strategy.

Your haircut and facial hair are among the most visible signals you send about how much attention you pay to yourself. A sharp cut communicates intention. An unkempt one communicates the opposite, regardless of how good everything else looks.

Finding your cut: The right haircut is determined primarily by your face shape. Oval faces are versatile and can carry almost any style. Square and rectangular faces benefit from cuts that add width on the sides. Round faces suit cuts with height. Heart-shaped faces are flattered by styles with more volume at the jaw line. If you are unsure of your face shape, photograph yourself straight-on and compare. AI tools, barbers, and stylists can all assist — come in with reference photos of cuts you find appealing to eliminate guesswork on both sides.

Facial hair: Entirely personal preference, but whatever you choose, maintain it. A well-kept beard signals intentionality. Define your neckline and cheek lines cleanly. If you shave, shave well — dry skin and razor burn undercut everything else.

Body hair: Personal preference. At minimum, keep the neck, shoulders, and ears well-managed — these are the most visible areas and the easiest to overlook.

Nails: Short, clean, and filed. Costs nothing and is noticed more than most men realize, particularly in professional and social settings.

Fragrance: A well-chosen fragrance is memorable in the best possible way. Two or three quality bottles — a fresh daytime option, a warmer evening scent, and an optional seasonal choice — is more than enough. Apply to pulse points: wrists, neck, chest. Less is more. Never spray directly on clothing.

HAIR LOSS — CAUSES, PREVENTION & WHAT ACTUALLY WORKS

Hair loss is one of the most psychologically impactful aspects of male appearance and one of the most poorly understood. The looksmaxxing community discusses it constantly — and for good reason. The right interventions, started early, can preserve the vast majority of what you have. Started late, they can still recover meaningful ground. Ignored entirely, the outcome is determined by genetics alone.

Understanding the cause — DHT and androgenetic alopecia

The overwhelming majority of male hair loss — roughly 95% — is androgenetic alopecia (AGA), commonly called male pattern baldness. The mechanism: an androgen called dihydrotestosterone (DHT), converted from testosterone by the enzyme 5-alpha-reductase, gradually miniaturizes hair follicles in men who are genetically predisposed to this sensitivity. Over time, affected follicles produce thinner, shorter, lighter hair until they stop producing hair entirely. The pattern typically follows the temples and crown first, though this varies.

The key insight: **this process begins years before it becomes visible**. By the time you notice thinning, significant miniaturization has already occurred. This is why early intervention is so disproportionately effective — you are protecting follicles that are still active rather than trying to revive ones that are already lost. A follicle that has been dormant for several years rarely responds to treatment. One that has been miniaturizing for one to two years often does.

The treatment hierarchy — from most to least evidence-backed

1. Finasteride — the foundational pharmaceutical intervention

Finasteride (brand name Propecia) is an oral 5-alpha-reductase inhibitor that blocks the conversion of testosterone to DHT, reducing scalp DHT by approximately 70%. It is the only oral medication FDA-approved for male pattern hair loss and has the most extensive clinical evidence base of any hair loss treatment. Studies show it halts progression in approximately 90% of men and produces visible regrowth in a significant proportion. Standard dose is 1mg daily. Results are typically visible at 6–12 months and continue improving with sustained use.

The side effect discussion deserves honesty: sexual side effects (reduced libido, erectile changes) are reported in a minority of users — approximately 2–4% in clinical trials, though community-reported rates vary. These effects are reversible upon discontinuation in the vast majority of cases. Topical finasteride (applied directly to the scalp rather than taken orally) is increasingly available through compounding pharmacies and online clinics, and shows meaningful efficacy with significantly lower systemic DHT suppression — a useful option for those concerned about systemic effects. This is not medical advice — discuss with a qualified physician.

2. Dutasteride — more potent, increasingly preferred

Dutasteride blocks both type I and type II 5-alpha-reductase (finasteride only blocks type II), suppressing scalp DHT by approximately 90% compared to 70% for finasteride. Multiple head-to-head studies have demonstrated superior hair count increases and faster results with dutasteride versus finasteride, with comparable or in some studies lower rates of side effects. It is FDA-approved for benign prostatic hyperplasia and approved specifically for AGA in South Korea, Japan, and Taiwan. In the US and UK it is used off-label for hair loss, accessible through online dermatology services. Standard dose is 0.5mg daily. Due to its longer half-life (5 weeks vs. 6–8 hours for finasteride), DHT suppression can persist for months after stopping — factor this in when planning a protocol.

3. Minoxidil — the topical foundation

Minoxidil (brand name Rogaine) is the other FDA-approved hair loss treatment and works through a completely different mechanism — vasodilation of scalp blood vessels, increasing blood flow and nutrient delivery to follicles, and directly prolonging the anagen (growth) phase of the hair cycle. It does not address DHT. This means it is highly complementary to finasteride or dutasteride rather than an alternative. Used together, finasteride plus minoxidil consistently outperforms either alone in clinical research, with one meta-analysis showing a 29.68 hairs/cm² increase at 24 weeks for the combination versus either agent individually.

5% topical minoxidil is available over the counter in foam and liquid forms. Apply to a dry scalp twice daily (once daily for the foam formulation). Results begin at 3–6 months. Low-dose oral minoxidil (0.625–2.5mg daily) is an increasingly popular option that reaches the scalp systemically and is often better tolerated by those who find topical application inconvenient — available by prescription through online clinics.

The most effective non-surgical protocol: finasteride (or dutasteride) + minoxidil (topical or oral) + microneedling. Each works through a different mechanism. Combined, they address DHT suppression, follicle stimulation, blood flow, and absorption simultaneously.

4. Microneedling / Derma Stamping — the amplifier

Microneedling (using a derma roller or derma stamp on the scalp) has become one of the most discussed hair loss tools in the looksmaxxing community, and the clinical evidence supports the enthusiasm. It works through two mechanisms: first, creating micro-injuries that trigger wound-healing growth factors (including VEGF and Wnt/ β -catenin pathway activation) which stimulate follicle activity; second, dramatically increasing the absorption of topically applied treatments — studies show microneedling can increase minoxidil absorption by up to 80%, making it significantly more effective than topical application alone.

For at-home use, a 0.5mm–1.0mm needle depth is appropriate for the scalp. Stamp (don't roll) over thinning areas, covering the entire zone systematically. Apply minoxidil or a peptide serum immediately after — the micro-channels created allow active ingredients to reach the follicle at concentrations not achievable through normal topical application. Session frequency: once per week, allowing the scalp to

fully heal between sessions. Use a clean, sterilized stamp each session and replace the needle cartridge regularly.

5. Peptides for hair — the emerging layer

Several peptides have demonstrated meaningful hair-supportive effects and are increasingly incorporated into advanced protocols, either stacked with the pharmaceutical approaches above or used independently by those who prefer non-hormonal options:

- **GHK-Cu (copper peptide):** The same compound covered in the skin section has documented effects on hair follicle stimulation — it increases VEGF (vascular endothelial growth factor) and improves scalp microcirculation, creating a more favorable environment for follicle activity. One 2023 study reported GHK-Cu triggered follicle growth at day 6 versus day 9 for 5% minoxidil in a head-to-head comparison. Combined with microneedling, a 2024 study showed 32% greater density increase versus peptide alone. Applied topically or injected subcutaneously — topical application immediately post-microneedling is the most practical approach.
- **Redensyl:** A biomimetic peptide complex that targets hair follicle stem cells, activating them from the resting phase back into active growth. Pilot double-blind data exists. Clinical trials report up to 214% increase in anagen (growth phase) activity. Available in topical serums, often combined with Procapil and Capixyl. Used in the looksmaxxing community as a non-hormonal daily serum — layer after minoxidil has dried.
- **Procapil:** A combination of biotinoyl tripeptide-1, apigenin, and oleanolic acid. Strengthens follicle anchoring, inhibits local DHT activity, and improves scalp microcirculation. Clinical data shows 121% increase in hair density at 4 months. Commonly combined with Redensyl in serum formulas.
- **Capixyl (Acetyl Tetrapeptide-3):** Works similarly to Procapil, inhibiting DHT locally and reinforcing the follicle-anchoring extracellular matrix. Good supporting evidence in small human trials. A 24-week RCT showed outcomes comparable to 3% minoxidil in mild AGA. Widely available in combination serums.
- **PTD-DBM:** Activates the Wnt/ β -catenin pathway — a core regulator of hair follicle cycling. Research from Yonsei University showed complete follicle regrowth in animal models. Emerging in topical formulations. Less clinical data than GHK-Cu or cosmetic peptide complexes but mechanistically compelling and increasingly discussed in the community.

6. Hair transplantation — the permanent solution

When hair loss has progressed beyond what pharmacological and topical interventions can adequately address, or when a man wants a permanent, definitive outcome, transplantation is the answer. Modern transplant techniques, when performed by a skilled surgeon, produce results that are natural-looking and undetectable — the era of obvious 'plugs' is decades past.

There are two primary techniques:

- **FUE (Follicular Unit Extraction):** Individual follicles are harvested one at a time from the donor area (back and sides of the scalp) using a micro-punch tool of 0.8–1.0mm diameter, then transplanted to the recipient area. Leaves no linear scar — only tiny dot scars that are concealed by surrounding hair, even at short lengths. This is now the dominant technique, chosen by approximately 85% of patients. Typical cost: \$7,000–\$15,000 in the US for 1,500–3,000 grafts. Significantly less in Turkey and Eastern Europe — though due diligence on surgeon quality is critical when traveling for surgery.

- **FUT (Follicular Unit Transplantation / Strip):** A strip of scalp is removed from the donor area and follicles are dissected from it, then implanted in recipient sites. Leaves a linear scar that is concealed by longer hair but visible if the head is shaved. Generally lower cost than FUE and can yield more grafts per session. Still the better option in some cases — a good surgeon will advise based on your specific hair loss pattern and donor supply.

What to expect from a transplant: Transplanted hair sheds within 2–4 weeks of surgery (this is normal — the follicles remain). Regrowth begins at months 3–4, is visible but thin at 6 months, and reaches full thickness and natural appearance at 12–18 months. The transplanted hair is permanent — it is taken from the DHT-resistant donor zone and retains that resistance in its new location. Native hair in surrounding areas can continue to thin over time, which is why maintaining a pharmaceutical protocol (finasteride/dutasteride + minoxidil) around a transplant is standard practice.

Critical considerations before proceeding:

- Get on finasteride or dutasteride before transplanting — hair loss progression must be stabilized first. Transplanting into an actively thinning field wastes grafts.
- Donor supply is finite. A skilled surgeon will plan conservatively to preserve donor hair for potential future procedures.
- Surgeon selection is everything. Research IAHRs (International Alliance of Hair Restoration Surgeons) members. Review actual patient results, not renderings. Avoid 'hair mills' that prioritize volume over precision.
- Turkey pricing (typically \$2,000–\$4,000 all-inclusive) has attracted many patients — outcomes can be excellent but the variance is enormous. If considering this route, research the specific surgeon as thoroughly as you would a local one.

The optimal protocol for most men: start finasteride or dutasteride plus minoxidil as early as possible (ideally at first signs of thinning). Add weekly microneedling with peptide serums applied immediately after. Reassess at 12 months. If results are satisfactory, maintain. If loss has progressed significantly, consult a transplant surgeon — with a stable foundation protocol in place, a transplant produces the best long-term outcome.

ANABOLIC COMPOUNDS & HAIR LOSS — THE RISK HIERARCHY

A significant portion of the looksmaxxing community is also exploring anabolic compounds for body composition. This is a reality, and the intersection with hair loss is one of the most poorly understood areas — largely because the guys using these compounds are often the same ones trying to protect their hair. The following is a straightforward risk reference, not an endorsement of use.

The core mechanism: anabolic steroids raise circulating androgens dramatically. Many convert to DHT or are themselves DHT derivatives, binding directly to scalp androgen receptors and accelerating follicle miniaturization in men who are genetically predisposed. Genetics determine susceptibility — some men can run highly androgenic compounds with minimal hair impact; others notice shedding within weeks of starting. If male pattern baldness exists anywhere in your family — maternal side included — assume elevated sensitivity.

A critical nuance: **DHT-derived compounds cannot be meaningfully protected against by finasteride or dutasteride.** Those drugs work by blocking 5-alpha-reductase — the enzyme that converts testosterone to DHT. If a compound is already structurally a DHT analogue (Masteron, Winstrol, Proviron), there is no conversion step to block. The damage pathway is direct. This is why finasteride is far more useful on a testosterone-only cycle than on a Masteron or Winstrol cycle.

HIGHEST RISK — DHT derivatives and highly androgenic compounds

- **Trenbolone (Tren):** Androgenic rating 500 — ten times testosterone. Does not convert to DHT but binds to androgen receptors with extreme affinity, including those in scalp follicles. Widely considered the single most hair-aggressive compound available. Cannot be protected by 5-AR inhibitors. Genetic hair loss predisposition plus trenbolone is one of the most reliably destructive combinations for hairline retention.
- **Masteron (Drostanolone):** A pure DHT derivative. Does not aromatize, does not convert — it IS DHT-based from the start, binding directly to scalp androgen receptors with no intermediate step. Extremely hair-toxic in predisposed individuals. Popular in cutting and competition prep, which is precisely when it does the most hair damage. Finasteride provides no meaningful protection.
- **Winstrol (Stanozolol):** DHT derivative. Does not aromatize. Androgenic rating 30 on paper, but in practice widely reported as one of the more aggressively hair-damaging compounds — the structural DHT base bypasses the conversion pathway entirely. Commonly associated with rapid shedding even at moderate doses in sensitive individuals.
- **Proviron (Mesterolone):** DHT derivative used as an ancillary drug, often to boost free testosterone and suppress SHBG. Highly androgenic in scalp tissue. Many users do not realize it carries significant hair risk because it is used in smaller doses and framed as a 'support' compound rather than a primary anabolic.
- **Dianabol (Methandienone / DBol):** Androgenic rating 60, but converts readily to DHT and produces high systemic androgen load. One of the most commonly used compounds and one of the most commonly associated with hair shedding during first cycles. High estrogen conversion

adds water retention but does not protect the follicle.

- **Anadrol (Oxymetholone):** Highly androgenic and estrogenic. Rapid, aggressive hair loss reported commonly in predisposed users. The combination of DHT conversion and high systemic androgen activity makes it among the harsher options for hairline retention.
- **Halotestin (Fluoxymesterone):** Androgenic rating 1900. Rarely used due to extreme toxicity profile, but included here because it occasionally appears in advanced protocols. Among the most aggressively androgenic compounds ever synthesized. Catastrophic hair risk in any predisposed individual.

MODERATE RISK — testosterone base compounds

- **Testosterone (all esters — Enanthate, Cypionate, Propionate, Sustanon):** The baseline reference. Androgenic rating 100. Converts to DHT via 5-alpha-reductase in scalp tissue, which means finasteride and dutasteride ARE protective here — this is their primary use case. At TRT doses (100–200mg/week), hair impact is generally modest in all but the most sensitive individuals. At supraphysiological doses (400mg+/week), DHT conversion increases proportionally and hair risk rises meaningfully. The most hair-protective way to run testosterone is TRT-level dose combined with dutasteride.
- **Primobolan (Methenolone):** Often marketed as a 'hair-safe' compound due to its mild androgenic rating (~44–57). In reality, it is a DHT derivative and does carry hair risk in sensitive individuals, particularly at higher doses. Finasteride provides limited protection. Relative to Masteron or Winstrol it is milder, but 'milder' is not 'safe' for the follicle-sensitive.

LOWER RISK — for those where hair retention is a priority

- **Nandrolone / Deca-Durabolin (Nandrolone Decanoate):** Converts via 5-alpha-reductase not to DHT but to dihydronandrolone (DHN), which has significantly weaker binding affinity at scalp androgen receptors than DHT. Consistently identified as one of the lowest-hair-risk injectable compounds available. Note: nandrolone suppresses natural testosterone production substantially, which reduces the DHT that would otherwise be produced from endogenous test. This is part of why it is relatively hair-friendly, but it makes PCT and hormonal recovery more complex.
- **Equipoise / Boldenone Undecylenate (EQ):** Does not readily convert to DHT. Androgenic rating approximately 50. Generally considered one of the lower-risk compounds for hair, with community consensus aligning with its comparatively mild androgen profile at the scalp. Estrogenic activity is present but lower than testosterone.
- **Anavar (Oxandrolone):** A DHT derivative, but with a very low androgenic rating (24) in practice and widely reported as one of the mildest compounds for hair at moderate doses. Many users with significant genetic hair loss predisposition report tolerating Anavar at 40–60mg/day without pronounced shedding. It remains a DHT analogue and is not zero-risk, but it is the closest thing to a 'mild' oral anabolic for those prioritizing hairline preservation. Finasteride provides minimal protection (same DHT-derivative bypass issue) but the low androgenic binding activity reduces

scalp impact.

- **Oral Turinabol (4-Chlorodehydromethyltestosterone):** Structurally altered to have essentially zero androgenic activity at the scalp. Widely regarded as the most hair-neutral oral anabolic available. Provides anabolic effects with minimal androgenic burden. Not without other health considerations (liver stress as an oral), but for hair specifically, it is the lowest-risk oral option.

Protective strategies for those using anabolic compounds

- **Dutasteride over finasteride** for testosterone-based cycles — more complete DHT suppression at the scalp. Provides no protection against DHT-derivative compounds (Masteron, Winstrol, Tren) since there is no conversion to block.
- **Ketoconazole shampoo (1–2%):** An anti-fungal with documented anti-androgenic properties at the scalp. Used 2–3 times per week, it reduces local scalp DHT activity and inflammation. A low-cost, low-effort add-on that provides meaningful scalp protection during a cycle. Leave on for 2–3 minutes before rinsing.
- **Minoxidil throughout the cycle:** Keeps follicles in the growth phase actively and counteracts some of the cycle-driven telogen shedding. Non-negotiable for anyone with hair loss predisposition running any anabolic compound.
- **Microneedling + peptide serums weekly:** Maintains active follicle stimulation and growth factor signaling during the period of elevated androgenic stress.
- **Choose your compounds deliberately:** If hair retention is a genuine priority, build cycles around lower-androgenic compounds (Nandrolone, EQ, Anavar, Turinabol) and keep testosterone at TRT-level. Avoid DHT derivatives entirely if hair loss predisposition is present. The muscle gains from Masteron or Winstrol are not worth an irreversible hairline to most men — and follicles lost to aggressive androgenic assault rarely return fully even with transplant intervention later.

The honest summary: there is no completely hair-safe anabolic compound. Every exogenous androgen carries some degree of risk in genetically predisposed individuals. The hierarchy above reflects relative risk, not absence of risk. The most effective hair protection strategy during any cycle is: dutasteride + minoxidil + ketoconazole shampoo + weekly microneedling, with compound selection tilted toward the lower-androgenic end of the spectrum. This is not medical advice — all of this should be discussed with a qualified physician.

04 — The Teeth

One of the most underestimated upgrades available.

Teeth are one of the most impactful aspects of attractiveness and are among the most frequently neglected. They don't need to be cosmetically perfect, but they need to be clean, reasonably white, and

as straight as possible.

Step one is professional cleaning. If you haven't been to the dentist recently, go. A deep clean alone produces a visible difference. Make it a twice-yearly habit.

Whitening: The author's personal recommendation, based on direct experience, is Crest Whitestrips. They are consistently effective, accessible, and outperform most of the more elaborate alternatives available at a higher price point.

Alignment: If your teeth are noticeably crooked, addressing it is worth the investment. Clear aligner systems have become significantly more advanced. Some alignment issues can be corrected with custom-molded retainers worn primarily at night. Consult your dentist to determine which category your situation falls into.

Priority order: Clean first. Whiten second. Straighten third. Each step makes a meaningful difference — and they compound.

05 — Skin & Face

The canvas matters. Maintaining it doesn't have to be complicated.

Good skin is one of the clearest signals of health. The baseline is simple: cleanse, moisturize, protect from the sun. Everything beyond that is optimization.

Moisturizer: If you use nothing else, use this. Cetaphil is a reliable and well-regarded choice — it absorbs quickly, leaves no residue, and works for most skin types. Apply morning and evening.

SPF: Sun exposure is the single largest driver of premature skin aging. A daily moisturizer with SPF 30+ is an easy solution.

Skin tone: Spending more time outdoors is the obvious solution and has the bonus of improving your overall health and mood. Self-tanning products have improved substantially and can produce natural-looking results without sun exposure. UV tanning beds, used carefully, are another option.

Advanced — GHK-Cu: This copper peptide has drawn significant research attention for its effects on skin repair and collagen production at the cellular level. Users report meaningful improvements in skin texture and overall appearance within weeks of consistent use. Both topical and injectable versions are available. Full dosing and reconstitution information is in the Peptide Reference Chapter.

Advanced — Melanotan 2 (MT2): MT2 works through the melanocortin system to stimulate your body's natural melanin production, producing a tan that goes well beyond what UV exposure alone can achieve. It is commonly used in combination with minimal UV exposure to accelerate and deepen

pigmentation. Common side effects include temporary darkening of existing moles or spots, increased libido, and initial nausea at higher starting doses (which is why low-and-slow titration matters). Full reference in the Peptide Reference Chapter.

Blemishes and skin conditions: Acne, redness, and uneven texture can all be meaningfully improved with the right products or treatments. A dermatologist visit is worth it if over-the-counter approaches aren't producing results.

06 — Posture & Physical Presence

How you carry yourself is as visible as what you wear.

Posture is one of the most overlooked elements of physical presentation, and one of the highest-leverage changes you can make at zero cost. Poor posture shrinks you — literally and figuratively. It undermines the effect of every other investment you make.

Stand tall, shoulders back and relaxed, chin parallel to the ground. Walk with intention. Make eye contact. These are not abstract soft-skill suggestions — they are physical signals that communicate confidence and health, and they register immediately.

If you spend significant time at a desk, active steps to correct posture are worthwhile. Stretching the chest and hip flexors, strengthening the posterior chain in your workouts, and adjusting your workstation setup all contribute meaningfully.

Advanced — Semax: Semax is a nootropic peptide derived from ACTH that enhances BDNF (brain-derived neurotrophic factor) activity, supporting focus, mental clarity, and cognitive performance. It is not a physical appearance compound, but the way a man presents himself — his engagement, sharpness, and presence in conversation — is very much part of the overall picture. Men who are mentally sharp and present simply come across better. For those interested, full dosing reference is in the Peptide Reference Chapter.

Slow down your movements. Men who are confident don't rush their gestures. They take up appropriate space. This is something you can begin practicing immediately.

07 — Facial Structure & Exercises

The free toolkit most guys don't know they already have.

This section is one of the most searched topics in the entire looksmaxxing space — and for good reason. The techniques here cost nothing, require no equipment, and can be done anywhere. They

also happen to be the area with the most misinformation attached to them, so it's worth being clear about what actually does something versus what is internet mythology.

The honest framing: these are long-game tools. None of them reshape your face overnight. What they do — done consistently over months and years — is move the needle on muscle definition, resting facial tension, posture, and in some cases structural development, particularly in younger men whose facial bones are still more responsive to mechanical force. If you are older, temper your expectations on the structural side, but do not dismiss these — the muscle tone and posture benefits apply to everyone.

Mewing — Tongue Posture

Mewing is the foundational technique of the entire facial exercise space, named after Dr. John Mew, a British orthodontist who developed the concept of orthotropics — the idea that oral posture significantly influences facial development. The basic premise: most people rest their tongue on the floor of their mouth, which provides no upward support to the palate. Correct tongue posture means resting the entire tongue — tip, middle, and back — flat against the roof of the mouth at all times.

Done consistently, this maintains upward pressure on the maxilla (upper jaw), which advocates argue can gradually widen and project the midface forward. The downstream effects theorized include higher cheekbones, deeper-set eyes, a more defined jawline, and reduction of a double chin through engagement of the platysma muscle. The clinical evidence for bone remodeling in fully grown adults is limited and contested — most orthodontists are skeptical. What is agreed upon by most dental professionals is that correct tongue posture improves nasal breathing, reduces mouth breathing (which has its own negative effects on facial structure over time), and supports better oral health. Even if the structural ceiling is lower than the community suggests, the correct resting position is simply proper oral posture — there is no downside to doing it.

Correct mewing technique: entire tongue flat against the roof of the mouth, lips sealed, teeth lightly touching or just apart, breathing through the nose. The back third of the tongue is the hardest part to get up — it should touch the soft palate. Do not press forcefully. This is a resting posture, not an exercise.

Jawline — Chewing and Masseter Development

The masseter is the primary jaw muscle and one of the most visible muscles in the face. A well-developed masseter widens and sharpens the lower face, creating the square jawline associated with masculine facial structure. This is one of the most evidence-supported areas of facial exercise — muscles respond to resistance training the same way everywhere in the body.

The most effective methods are also the simplest. Chewing harder foods — raw vegetables, tough cuts of meat, dense bread — naturally engages the masseter more than a diet of soft processed food. For deliberate training, mastic gum (a natural resin gum from the Mediterranean, significantly harder than

regular chewing gum) has become the most widely used tool in this space. Chew for 20–30 minutes per day to start, alternating sides. Results in masseter size are visible within several months of consistent use. Specialized jaw training devices also exist for those who want to add more progressive resistance, though mastic gum alone is sufficient for most.

Important: Do not overtrain the jaw. TMJ (temporomandibular joint) dysfunction is a real risk from excessive or one-sided jaw loading. Train both sides equally, rest if you feel joint pain (not muscle soreness), and build volume gradually.

Hunter Eyes — Canthal Tilt and the Eye Area

Hunter eyes is the community term for an eye appearance characterized by: a positive or neutral canthal tilt (outer corners of the eyes level with or slightly higher than the inner corners), minimal upper eyelid exposure, a subtle hooding of the upper lid, a prominent brow ridge, and deep-set orbital structure. The opposite — round, wide-open eyes with a negative canthal tilt — is called prey eyes in community terminology.

The structural elements of hunter eyes are largely genetic and skeletal. The orbital bones, brow ridge projection, and canthal tilt are primarily determined by your underlying facial structure. That said, several soft tissue and behavioral factors are within your control:

- **Resting gaze and eyelid tension:** Many men habitually open their eyes wider than necessary, raising the upper lid and exposing more of the sclera (white). Practicing a more relaxed, slightly hooded resting gaze — without squinting or looking tired — immediately creates a more intense, controlled appearance. This is partly a habit and partly muscle control. Practice in a mirror until it becomes your natural default.
- **Body fat and puffiness:** Periorbital fat (the fat around and under the eyes) is significantly affected by overall body fat percentage and water retention. Lower body fat, adequate sleep, and reduced sodium intake all reduce puffiness around the eye area and can meaningfully sharpen the appearance of the orbital region.
- **Mewing's indirect effect:** Advocates argue that long-term correct tongue posture gradually lifts the midface upward, which can increase the structural support around the eyes and contribute to a more hooded lid appearance over years of practice. This is the most plausible structural mechanism for non-surgical canthal tilt improvement.
- **Eyebrow grooming:** The shape and position of the brow has a major impact on how the eye area reads. A well-groomed, slightly lower-set, straighter brow (as opposed to a highly arched one) enhances the hunter eye aesthetic significantly. This is an immediate, zero-cost adjustment.
- **Cold therapy:** Applying cold (ice packs, cold water, chilled eye masks) to the orbital area in the morning reduces puffiness and tightens the skin around the eyes. Consistent use makes a visible difference in under-eye definition.

Neck Training

The neck is one of the most overlooked parts of facial aesthetics and one of the most impactful. A thick, developed neck frames the face completely differently than a thin one. It also signals physical strength and health in a way that is difficult to fake. Neck training is standard in combat sports for this reason — it is among the most visible muscular developments on the body.

Direct neck training can be done with bodyweight or light resistance: neck flexion, extension, and lateral resistance holds. Wrestlers' bridges (where you support your body weight on your head and neck in a back-bridge position) are the classic high-volume approach but require care and progression. A simpler starting point is manual resistance with your own hand — press against your forehead, the back of your head, and each side while resisting with your neck for 3–5 sets of 10–15 seconds in each direction. Shrugs and trap development from deadlifts also contribute to the overall neck-and-shoulder silhouette.

Nasal Breathing and Mouth Taping

Chronic mouth breathing is documented to have negative effects on facial development, particularly in younger people — it contributes to a longer, narrower face, weaker jaw development, and poor tongue posture. Correcting to nasal breathing is part of the mewing foundation and has independent health benefits including better sleep quality, improved oxygen efficiency, and reduced snoring.

Mouth taping during sleep — using a small piece of medical or surgical tape over the lips to encourage nasal breathing — has become popular in this community. The evidence base is modest but growing, and the practice is low-risk for most people. If you wake up with a dry throat or frequently mouth-breathe at night, it is worth exploring.

What to Avoid

One technique that appears in this community and must be named specifically to warn against it: **bonesmashing** — deliberately striking facial bones with hard objects to trigger bone remodeling. This is dangerous. Facial surgeons have documented risks including fractures, facial misalignment, nerve damage, and vision impairment. There is no credible evidence it produces the intended results. Avoid entirely.

The hierarchy for this section: Mewing and nasal breathing first — these are foundational and have no downside. Jaw training second — measurable results with moderate effort. Neck training third — high visual impact, underutilized. Eye area habits and grooming fourth — immediate results, zero cost. Everything else is optimization on top of these.

08 — The Skincare Stack

Most men do nothing. A small routine puts you ahead of almost everyone.

The skin section earlier in this guide covered the basics — moisturizer, SPF, skin tone. This section goes deeper, because the community is right about one thing: skincare has an exceptionally high return on investment relative to the effort required, and most men are leaving significant gains on the table by either doing nothing or doing it wrong.

The goal is not complexity. The goal is a small number of the right products, used consistently, in the right order. The difference between a man who has been doing this for a year and one who hasn't is visible and significant.

The Hierarchy — what actually moves the needle:

1. Sunscreen (SPF 30–50+, daily, non-negotiable)

UV damage is the single largest driver of premature skin aging — not genetics, not diet, not stress. It is UV. A broad-spectrum SPF applied every morning prevents the collagen breakdown, hyperpigmentation, and textural damage that accumulates invisibly for years before it becomes visible all at once. If you do nothing else in this section, do this. Apply after moisturizer, every morning, 365 days a year. La Roche-Posay Anthelios is widely regarded as one of the best formulations — lightweight, no white cast, and effective. EltaMD UV Clear is another strong option particularly for acne-prone skin.

2. Tretinoin — the gold standard

Tretinoin (all-trans retinoic acid, brand name Retin-A) is the single most evidence-backed topical skincare compound available. Originally developed for acne, it was quickly recognized as the most effective anti-aging topical in existence. It works by accelerating skin cell turnover, stimulating collagen production, reducing hyperpigmentation, clearing pores, and smoothing texture — simultaneously. Nothing else topical comes close to its documented efficacy across this range of outcomes.

It requires a prescription in most countries (US, UK, Australia, most of Europe). Online dermatology services — Hims, Keeps, Curology, and others — have made this dramatically more accessible in recent years. A prescription typically costs less than most over-the-counter serums claiming to do a fraction of what tretinoin actually does.

Start at the lowest concentration available (0.025%) and use it every other night for the first 4 weeks. Expect a purge period — skin may get temporarily worse as trapped sebum and dead cells are pushed to the surface faster than usual. This is normal and resolves. Push through it. After 8–12 weeks, the improvement in texture, tone, and clarity is typically substantial and continues to compound with consistent use.

Tretinoin application rule: cleanse → wait 20 minutes for skin to fully dry → apply a pea-sized amount to entire face → moisturize on top. Never apply to damp skin — it dramatically

increases irritation. Always follow with SPF the next morning. Never use on the same night as AHA/BHA exfoliants.

Adapalene (Differin 0.1%) — the OTC alternative

If tretinoin is unavailable or you want to start more conservatively, adapalene is available over-the-counter in the US and increasingly elsewhere. It is a retinoid that works similarly to tretinoin with less irritation. Research has shown it comparable to tretinoin 0.05% for acne and photoaging improvement. A strong starting point for anyone new to retinoids.

3. The Daily Routine — morning and evening

- **Morning:** Gentle cleanser → Vitamin C serum (brightening, antioxidant protection, collagen support) → Moisturizer → SPF 30–50+
- **Evening:** Double cleanse (oil/balm first to remove SPF, then gentle cleanser) → Tretinoin or retinoid (on dry skin) → Moisturizer
- **2–3× per week (not on tretinoin nights):** Chemical exfoliant — AHA (glycolic or lactic acid) for texture and tone, or BHA (salicylic acid) for pores and oily/acne-prone skin. Never use on the same night as tretinoin.

4. Upgrades worth considering

- **Niacinamide (5–10%):** Reduces pore appearance, controls oil, improves skin barrier, reduces redness. Extremely well-tolerated and can be used morning or evening. One of the best-value active ingredients available.
- **Hyaluronic acid:** Applied to damp skin before moisturizer, draws water into the skin. Useful if you find tretinoin drying. Must be applied to damp skin and sealed with moisturizer — applied to dry skin in low-humidity environments it can actually dehydrate.
- **Vitamin C serum:** L-ascorbic acid at 10–20% concentration is the most effective form. Brightens, protects against UV damage (in addition to SPF, not instead of), and supports collagen synthesis. Use in the morning, apply before SPF.

5. What to avoid

- Physical scrubs and walnut/apricot exfoliants — these create micro-tears in the skin. Use chemical exfoliants only.
- Overloading actives — stacking tretinoin, AHAs, BHAs, and vitamin C on the same night destroys your skin barrier. Build slowly, one active at a time. The skin barrier repair is what allows everything else to work.
- Skipping SPF after tretinoin — tretinoin makes skin significantly more photosensitive. SPF the morning after is non-negotiable.

Timeline: 4–6 weeks to see initial improvements in hydration and glow. 2–3 months for noticeable texture and tone changes. 6–12 months for significant anti-aging and scar improvement. Take monthly progress photos. Consistency with a simple routine beats an elaborate one you don't follow.

09 — Debloating & Face Definition

The fastest visible improvement most men can make — often within days.

This is one of the most discussed topics in the looksmaxxing community for good reason: the difference between a bloated face and a defined one can be dramatic, and the interventions that produce it are free, immediate, and available to anyone. Most men walking around with a soft, undefined face are not carrying excess fat — they are carrying excess water. These are fundamentally different problems with different solutions.

Understanding the distinction is critical. **Bloat is water retention**, caused primarily by electrolyte imbalance — too much sodium relative to potassium. **Face fat** is actual adipose tissue that requires caloric deficit and time to reduce. **Lymphatic congestion** is fluid that has accumulated in facial tissue and can be manually moved. Conflating these three leads to confusion and wasted effort.

The Sodium-Potassium Balance — the core lever

Sodium causes your body to retain water. Potassium encourages your body to flush it. The modern diet is chronically high in sodium and low in potassium, which means most men are operating with a baseline of facial puffiness that they have come to accept as their normal. It is not their normal — it is dietary. The target sodium-to-potassium ratio is approximately 1:4. In practical terms: reduce processed foods, restaurant meals, and packaged snacks (the primary sodium sources), and increase high-potassium foods.

- **High-potassium foods:** Bananas, avocados, potatoes, dried apricots, coconut water, spinach, salmon, white beans. Coconut water in particular has become a community staple — high potassium, low calorie, fast-acting.
- **Immediate debloat protocol:** Reduce sodium to under 1,500mg for 48–72 hours, increase water intake significantly, load potassium-rich foods. Most men report visible facial definition change within 2–3 days.
- **Avoid:** Carbonated drinks (gas traps in tissue), alcohol (vasodilates blood vessels, causes inflammatory retention, dehydrates then triggers compensatory water holding), excess refined carbohydrates (each gram of glycogen binds 3–4 grams of water — a high-carb day shows in the face).

Hydration — the counterintuitive piece

Drinking more water when you are retaining water seems backwards. It is not. When the body is chronically under-hydrated, it holds onto every drop it gets as a survival response. Adequate hydration — at least 2.5–3 litres per day, more if training — signals to the body that water is abundant and retention is unnecessary. Add electrolytes (not just sodium — magnesium and potassium specifically) to your water, particularly post-training.

Lymphatic Drainage — gua sha and manual massage

The lymphatic system does not have a pump — it relies on movement and manual pressure to circulate. Fluid that accumulates in facial tissue, particularly overnight, can be moved through lymphatic drainage massage and gua sha. This is not a myth — it is basic physiology, and the results are visible.

Gua sha technique: apply a facial oil first (any light oil works). Using a gua sha tool or your knuckles, work in upward and outward strokes — from the center of the face outward toward the ears, and from the jaw down toward the collarbone where the lymph nodes drain. Use light-to-moderate pressure. Do this for 3–5 minutes each morning, ideally after cold therapy. The combined effect of cold (which constricts vessels and reduces inflammation) followed by lymphatic drainage is the fastest facial definition protocol available without any products or compounds.

Important: lymphatic drainage moves fluid temporarily. It is not fat loss. It does not change bone structure. What it does is reveal the structure you already have by clearing the fluid sitting on top of it. Think of it as daily maintenance, not a transformation.

Sleep position

Sleeping face-down compresses facial tissue for 6–8 hours and dramatically increases morning puffiness. Sleeping on your back, ideally with your head slightly elevated (an extra pillow), allows overnight fluid to drain rather than accumulate. This single change produces a noticeably less puffy face within a week for most people who make it. If you are a side sleeper, a silk or satin pillowcase reduces compression and friction on the skin compared to cotton.

Alcohol

Alcohol deserves its own callout because its effect on facial definition is severe and multi-mechanism: it dehydrates (triggering compensatory water retention), causes vasodilation (puffiness and redness), triggers an inflammatory response, elevates cortisol, and disrupts sleep quality — all of which show directly in the face the next day. A single heavy night can undo a week of debloating progress. This is not a moral statement — it is a practical one. If facial definition is a priority, alcohol is the single biggest dietary obstacle most men face.

Cortisol and face fat

Chronically elevated cortisol specifically drives fat storage in the face and around the midsection. High cortisol = higher sodium retention, more inflammatory response, worse sleep, worse insulin sensitivity.

Everything that affects cortisol — sleep quality, stress, training recovery, alcohol — shows in the face. Managing cortisol is not optional if facial definition is the goal. The sleepmaxxing section covers this in detail.

Actual fat loss — when bloat is not the issue

If you have addressed the above and your face is still not as defined as you want, the remaining variable is actual subcutaneous fat. The face cannot be spot-reduced — fat loss is systemic. Getting to a lean body fat percentage (roughly 12–15% or below for most men) is when facial bone structure begins to emerge fully. Caloric deficit plus resistance training is the mechanism. The GLP compounds covered in the peptide reference chapter are the advanced tool for this — but they work within the same fundamental framework.

The quick-start debloat protocol: for 72 hours, cut sodium dramatically, cut alcohol entirely, drink 3L of water with electrolytes daily, eat high-potassium foods at every meal, sleep on your back, do 5 minutes of lymphatic drainage each morning. Most men see a visible difference within 48 hours. This is the fastest free improvement available.

10 — Sleepmaxxing

The highest-ROI looksmaxxing habit. Free, requires no equipment, amplifies everything else.

Sleep is not a passive state. It is the period during which your body executes almost every repair, recovery, and hormonal process that determines how you look, feel, and perform. Shortcut it, and every other investment in this guide produces a fraction of its potential return.

The community has coined the term **sleepmaxxing** to describe the deliberate optimization of sleep quality — not just duration, but architecture, timing, and the hormonal environment that sleep controls. This is not about getting 8 hours. It is about getting the right 8 hours, structured correctly, at the right time.

What sleep controls — the hormonal cascade

- **Testosterone:** Sleeping 5 hours instead of 8 reduces testosterone by 10–15% in documented studies. Lower testosterone means less muscle, more fat storage, worse skin quality, lower drive. A single week of poor sleep produces measurable hormonal decline.
- **Human Growth Hormone (HGH):** Released almost exclusively during deep sleep (slow-wave sleep), primarily in the first half of the night. HGH drives tissue repair, muscle growth, fat metabolism, and skin regeneration. Miss deep sleep, miss HGH. No supplement adequately replaces this.

- **Cortisol:** Poor sleep keeps cortisol chronically elevated. High cortisol breaks down collagen, drives facial fat storage, suppresses testosterone, impairs skin barrier function, and accelerates aging. Sleep is the primary cortisol regulation mechanism.
- **Skin barrier:** Sleep deprivation impairs the skin's ability to retain moisture and repair daily damage. The result is drier, duller skin that ages faster. Studies have shown sleep-deprived individuals are rated significantly less attractive and less healthy-looking after just two nights of poor sleep.

Circadian rhythm — the foundation everything else rests on

Your circadian rhythm is your body's master clock, governed primarily by light exposure. Getting this right is the single highest-leverage intervention in sleep optimization — and it costs nothing.

- **Morning sunlight within 30 minutes of waking:** This is the most important single habit in this section. Natural sunlight through the eyes (no sunglasses, no windows — direct outdoor light) in the first 30 minutes after waking suppresses residual melatonin, triggers the cortisol awakening response (a healthy, timed cortisol spike that sets your energy for the day), regulates your circadian clock, and directly improves sleep quality that night. 10–15 minutes is sufficient. This single habit improves testosterone, sleep depth, energy, and mood simultaneously. It is free and takes less than 15 minutes.
- **Consistent wake time:** Your body's internal clock is set by when you wake up, not when you go to sleep. A consistent wake time — even on weekends, within 30 minutes — is more important than sleep duration. Irregular wake times fragment the circadian rhythm and impair every hormonal process that depends on it.
- **Block artificial light after dark:** Your brain interprets any light as daylight and suppresses melatonin accordingly. Overhead lights, screens, and phones after sunset delay sleep onset and reduce sleep quality. Blue-light blocking glasses after 8pm or switching to dim, warm lighting makes a measurable difference. Avoid screens in the 30 minutes before bed.

The sleep environment

- **Temperature:** Core body temperature must drop to initiate and maintain sleep. The optimal room temperature is 65–68°F (18–20°C). Cold room, warm blanket is the formula. Training or showering hot within 2–3 hours of bed raises core temperature and delays sleep onset.
- **Darkness:** Complete darkness — blackout curtains or a sleep mask. Even small amounts of light through closed eyelids suppress melatonin and reduce sleep depth.
- **Silence or consistent sound:** Sudden noise disrupts sleep stages. White noise, brown noise, or earplugs address this if your environment is variable.
- **Stop eating 2–3 hours before bed:** Digestion raises core temperature and keeps metabolism active, both of which impair sleep architecture. The body should be in recovery mode during sleep, not processing food.

Pre-sleep protocol

- **No intense training within 3 hours of bed:** Intense exercise elevates cortisol and core body temperature. Morning or early afternoon training is substantially better for sleep quality than evening sessions.
- **Delay caffeine 90 minutes after waking:** Caffeine consumed immediately on waking interrupts your natural cortisol awakening response and creates an afternoon crash. Waiting 90 minutes allows natural cortisol to peak first, then caffeine extends the energy curve rather than replacing it.
- **Last caffeine before 2pm:** Caffeine has a half-life of approximately 5–6 hours. Coffee at 4pm means half of it is still active at 9–10pm, directly impairing sleep onset and depth.

Supplements that actually help

- **Magnesium glycinate (200–400mg, 30–60 minutes before bed):** The most consistently useful sleep supplement. Promotes muscle relaxation, reduces cortisol, supports GABA activity (the calming neurotransmitter). Glycinate form is preferred — well-absorbed and gentle on digestion. Avoid magnesium oxide (cheap, poorly absorbed).
- **L-Theanine (100–200mg):** Promotes relaxation without sedation. Useful for calming racing thoughts without affecting sleep architecture. Can be combined with magnesium.
- **Glycine (3g):** Amino acid that supports deep sleep quality and may reduce the time to fall asleep. Also has skin benefits — it is a collagen precursor.
- **Melatonin (0.5–1mg):** Use sparingly — for jet lag, travel, or occasional schedule resets. Nightly use can reduce your body's natural melatonin production over time. The dose most people take (5–10mg) is dramatically higher than needed. 0.5mg is physiologically appropriate for most people.

Sleep position — it matters for your face

As covered in the debloating section: sleep on your back with slight head elevation. Face-down or side sleeping compresses facial tissue for hours and consistently produces morning puffiness. If you cannot sleep on your back, a silk pillowcase significantly reduces the compression and friction effects on skin compared to cotton.

The compounding effect: one week of optimized sleep — consistent timing, morning sunlight, dark cool room, no late caffeine, magnesium before bed — produces visible improvements in skin quality, facial definition, energy, and body composition. Most men notice the difference before two weeks. None of this costs anything meaningful. It is the highest-ROI item in this entire guide.

11 — Cortisol Management

The silent saboteur. Chronically elevated cortisol undermines everything else in this guide.

Cortisol is your body's primary stress hormone, produced by the adrenal glands in response to perceived threats. In short, controlled bursts it is essential — it gets you out of bed, sharpens focus in demanding situations, and fuels acute performance. The problem is modern life produces chronic, low-grade cortisol elevation that never fully resolves. This state — cortisol dominance — is one of the most destructive forces working against everything this guide is trying to build.

The community at looksmax.org has a phrase for it: **cortisol dominance nullifies the returns of your skincare, training, diet, and recovery protocols**. That is not an exaggeration. Every chapter of this guide is undermined by chronically elevated cortisol. Understanding what it does and how to manage it is not optional.

What chronically elevated cortisol does to your appearance

- **Collagen breakdown:** Cortisol directly inhibits fibroblast activity — the cells responsible for producing collagen and elastin. Chronic elevation accelerates skin thinning, wrinkling, and loss of firmness. This is the hormonal mechanism behind why chronically stressed people age faster visibly, not just physiologically.
- **Facial fat redistribution:** Chronically high cortisol drives fat storage specifically in the face (particularly around the cheeks and under the chin) and the midsection. It does this through increased insulin resistance and direct lipogenic signaling. This is the hormonal face fat that no amount of sodium restriction or lymphatic drainage will fully resolve — the cortisol source must be addressed.
- **Testosterone suppression:** Cortisol and testosterone exist in a direct inverse relationship. Chronically elevated cortisol suppresses LH (luteinizing hormone) production, which reduces testosterone synthesis. Lower testosterone means less muscle, more fat, worse skin quality, reduced drive, and weaker facial masculinity over time.
- **Skin barrier impairment:** High cortisol reduces the skin's ability to retain moisture and repair daily environmental damage. The result is chronic dryness, increased sensitivity, slower healing, and accelerated photoaging — even in men with otherwise solid skincare routines.
- **Hair loss acceleration:** Cortisol promotes telogen effluvium — stress-induced shedding where follicles are pushed prematurely into the resting phase. It also suppresses the growth factors (including IGF-1 and VEGF) that support active follicle cycling. Any man doing a hair protocol while chronically stressed is working against himself.
- **Sleep destruction:** Cortisol and melatonin are inversely regulated. Elevated evening cortisol — the most common pattern in chronically stressed individuals — directly suppresses melatonin production, delays sleep onset, reduces deep sleep duration, and tanks HGH release. Every negative effect of poor sleep covered in the Sleepmaxxing chapter is downstream of cortisol dysregulation.

- **Skin inflammation and breakouts:** Cortisol upregulates inflammatory cytokines and increases sebum production. The result is increased acne frequency and severity, persistent redness, and a generally inflamed skin appearance that topical skincare alone cannot fully correct.

The cortisol rhythm — understanding the baseline

Cortisol is not inherently bad. It follows a natural diurnal rhythm: high in the morning (the cortisol awakening response, which sets energy and alertness for the day), declining through the afternoon, and low by evening to allow sleep. The goal is not to eliminate cortisol — it is to restore and maintain this natural rhythm. Chronic stress, poor sleep, overtraining, alcohol, and constant digital stimulation all flatten and dysregulate this curve, keeping levels inappropriately elevated throughout the day and night.

Lifestyle interventions — the highest-leverage tools

- **Morning sunlight:** As covered in the Sleepmaxxing chapter, natural light exposure within 30 minutes of waking anchors the cortisol awakening response at the correct time and magnitude. This single habit more than any supplement normalizes the daily cortisol curve. It is free and takes 10–15 minutes.
- **Training load management:** Resistance training acutely elevates cortisol — this is a normal, necessary part of the adaptive process. The problem arises with overtraining: training volume or frequency that exceeds recovery capacity keeps cortisol chronically elevated. Symptoms include persistent fatigue, declining performance, increased resting heart rate, and worsening sleep. More training is not always better. Recovery is where adaptation occurs.
- **Deliberate nasal breathing:** Slow diaphragmatic breathing at 4–6 breaths per minute activates the parasympathetic nervous system within minutes, directly opposing the cortisol stress response. Box breathing (4 counts in, 4 hold, 4 out, 4 hold) or the 4-7-8 method are structured approaches. Five minutes of deliberate slow breathing produces measurable reductions in cortisol and heart rate variability improvement — usable anywhere, at any time, at zero cost.
- **Walking — particularly outdoors:** 20+ minutes of low-intensity walking, especially in natural environments, reduces cortisol and amygdala hyperactivity (the brain region driving threat perception). An evening walk without a phone is one of the most effective daily cortisol-lowering habits available. It also improves sleep onset.
- **Eliminating unnecessary stressors:** Information overload — constant news, social media, and notifications — keeps the threat-detection system in a low-grade activated state all day. Limiting consumption, particularly in the first and last hours of the day, meaningfully reduces baseline cortisol load. This is not a lifestyle opinion — it is physiology.
- **Alcohol:** As covered in the Debloating chapter, alcohol elevates cortisol through multiple mechanisms and disrupts the cortisol rhythm for 24–48 hours after consumption. It is one of the most reliable cortisol elevators available and one of the most frequently overlooked.

- **Heat therapy:** A bath or sauna at around 40°C (104°F) for 15–20 minutes, 2–3 times per week, boosts GABA activity, reduces cortisol, and improves sleep quality when done in the evening. This is well-supported and has the added benefit of improving cardiovascular health and growth hormone release.

Supplements with genuine evidence

- **Ashwagandha (300–600mg standardized extract, daily):** The most extensively studied cortisol-reducing supplement available. A 2024 meta-analysis of 15 randomized controlled trials involving 873 participants confirmed statistically significant cortisol reduction and improvements in perceived stress. One landmark RCT found a 27.9% reduction in serum cortisol at 600mg/day over 60 days. Works by modulating the HPA axis — the central cortisol production pathway. Take in the morning or split AM/PM. Generally well-tolerated for up to 3 months; cycle off periodically.
- **Magnesium glycinate (200–400mg, evening):** Covered in the Sleepmaxxing chapter — magnesium supports nervous system regulation, reduces the cortisol response to stress, and improves sleep quality. The glycinate form is preferred for absorption and tolerability. Magnesium deficiency is widespread and directly impairs cortisol regulation — addressing it is foundational before anything else.
- **Phosphatidylserine (400–600mg, before training or stressful periods):** A phospholipid that blunts the cortisol spike in response to acute stress and exercise. Clinical trials show 20–35% reduction in cortisol area under the curve when taken before training. Particularly useful for men doing high-volume training who want to mitigate the cortisol cost of their sessions without reducing training intensity.
- **Rhodiola Rosea (200–400mg, morning):** An adaptogen that increases stress resistance rather than simply sedating the stress response — meaning it helps maintain function and performance under stress while reducing the hormonal cost. Best evidence is for burnout, fatigue, and cognitive performance under stress. Takes 2–4 weeks to reach full effect. Do not take in the evening as it has mild stimulatory properties.
- **L-Theanine (100–200mg):** Promotes calm alertness by increasing alpha brain waves without sedation. Reduces cortisol spikes from acute stressors. Pairs well with caffeine to smooth out the cortisol spike from coffee. Can be taken morning or evening.
- **Vitamin C (500–1000mg, daily):** The adrenal glands have among the highest vitamin C concentrations of any tissue in the body — they use it in cortisol synthesis and regulation. Supplementation supports adrenal function, reduces post-exercise cortisol, and has skin-brightening benefits covered in the skincare section.

What to track

Cortisol dysregulation has a recognizable symptom profile: persistent facial puffiness that does not respond to dietary correction, difficulty sleeping despite fatigue, recovery that seems slower than training volume warrants, mood volatility, central fat accumulation despite training, and skin that looks

inflamed or ages faster than expected. If several of these apply simultaneously, cortisol management should be treated as a priority intervention, not an afterthought.

Salivary cortisol testing (morning, afternoon, and evening samples) is available through several online services without a physician visit and provides an objective baseline to work from. If training load, sleep, and lifestyle interventions are already solid and the pattern persists, it is worth getting a full hormonal panel including cortisol, DHEA-S, and testosterone to assess the broader picture.

The stack for most men: morning sunlight daily, magnesium glycinate nightly, ashwagandha 300–600mg daily for 60-day cycles, phosphatidylserine before hard training sessions, deliberate breathing practice 5 minutes per day, evening walks without a phone, and strict alcohol reduction. This addresses cortisol from multiple directions simultaneously. The effects on sleep quality, skin appearance, facial definition, and training recovery are typically visible within 2–4 weeks of consistent implementation.

12 — Attitude

The force multiplier that makes everything else work harder.

This guide is titled Looksmaxxing, but it would be incomplete — and dishonest — without addressing this. Your attitude is visible. It shows in how you walk, how you make eye contact, how you respond when things don't go your way, and how you treat people who can do nothing for you.

Many if not most visual shortcomings are easily absorbed by a man who carries himself well and engages with genuine warmth and confidence. These men are routinely perceived as more attractive than men with objectively superior physical features who carry themselves poorly.

Confidence is not arrogance. It is the quiet certainty that you are working on yourself, that you bring value to interactions, and that you don't require external validation to feel settled. It is built through action — through keeping promises to yourself, through the discipline of the gym, through dressing with intention. It compounds.

Warmth costs nothing and returns everything. Smile genuinely. Listen more than you speak. Remember names. These behaviors have an outsized effect on how attractive and magnetic you are perceived to be.

The goal of all of this: to become the version of yourself that you would respect most. Looks are the starting point. The man behind them is the destination.

The peptide space is flooded with misinformation — wrong reconstitution ratios, misread syringe markings, and protocol advice copied from forum posts that were copied from other forum posts. The cards below are built from the most consistently cited research protocols and reconstitution standards for each compound, with the math worked out for you.

Every card follows the same format: what the compound is, how it works, how to reconstitute it correctly, what dose that translates to on a standard U-100 insulin syringe, and the commonly referenced protocol structure. None of this is a prescription or personal medical recommendation — it is educational reference material intended to help you understand and verify what you are looking at.

SYRINGE MATH EXPLAINED: A standard U-100 insulin syringe has 100 units per mL. So 10 units = 0.10 mL, 20 units = 0.20 mL, and so on. If your concentration is 5 mg/mL and your target dose is 0.5 mg, you draw 0.10 mL = 10 units. Always double-check your math before drawing. Mistakes at reconstitution compound through every subsequent dose.

RETATRUTIDE

Triple receptor agonist · Body composition · Weekly injection

PURPOSE

Body composition, fat loss, metabolic health. The most potent compound in the GLP class currently in trials — acts on GLP-1, GIP, and glucagon receptors simultaneously.

HOW IT WORKS

Triple agonism drives insulin secretion (glucose-dependent), appetite suppression, and elevated metabolic rate through glucagon receptor activity — the third pathway is what separates it from Tirzepatide.

RECONSTITUTION GUIDE · Add bacteriostatic water slowly down the inside vial wall — never directly onto powder. Swirl gently. Never shake.

VIAL SIZE	BAC WATER	CONCENTRATION
10 mg	1.0 mL BAC water	10 mg/mL
20 mg	2.0 mL BAC water	10 mg/mL
30 mg	3.0 mL BAC water	10 mg/mL

DOSE REFERENCE · Based on the most commonly cited research protocol. Assumes standard U-100 insulin syringe (1 mL = 100 units).

DOSE	AMOUNT	U-100 SYRINGE UNITS
Start (Weeks 1–4)	2 mg/week	20 units
Escalation	4–6 mg/week	40–60 units
Standard target	6–8 mg/week	60–80 units
Advanced	12 mg/week	120 units*

ROUTE

Subcutaneous injection

FREQUENCY

Once weekly · Same day each week · Rotate injection sites

CYCLE

Minimum 24 weeks · Clinical trials ran to 48 weeks

NOTES

*At 10 mg/mL, a 12 mg dose requires 1.2 mL — split across two injection sites. Titrate slowly; GI side effects nearly double when dose is jumped too quickly. Do not skip escalation steps.

COMMON SIDE EFFECTS

Nausea, vomiting (especially without titration), decreased appetite, injection site reactions, fatigue. GI effects typically reduce after first 4–6 weeks.

For informational/educational purposes only. Not medical advice. Not FDA-approved for human use. Consult a licensed physician before use. Research-use only compound.

MELANOTAN 2 (MT2)

Melanocortin agonist · Tanning · Skin pigmentation · Daily injection

PURPOSE

Stimulates melanin production to produce a deep, lasting tan with minimal UV exposure. Also known for significant libido-enhancing effects as a secondary mechanism.

HOW IT WORKS

Synthetic analog of alpha-MSH. Binds to melanocortin receptors (MC1R for pigmentation, MC4R for libido and appetite), triggering melanocyte activity to produce eumelanin — the dark pigment.

RECONSTITUTION GUIDE · Add bacteriostatic water slowly down the inside vial wall — never directly onto powder. Swirl gently. Never shake.

VIAL SIZE	BAC WATER	CONCENTRATION
10 mg	2.0 mL BAC water	5 mg/mL (5,000 mcg/mL)
10 mg	3.0 mL BAC water	3.33 mg/mL (3,333 mcg/mL)

DOSE REFERENCE · Based on the most commonly cited research protocol. Assumes standard U-100 insulin syringe (1 mL = 100 units).

DOSE	AMOUNT	U-100 SYRINGE UNITS
Starter (first 3 days)	100–250 mcg	2–5 units (at 5 mg/mL)
Loading daily dose	500 mcg	10 units (at 5 mg/mL)
Max recommended	1,000 mcg	20 units (at 5 mg/mL)
Maintenance (ongoing)	250–500 mcg	5–10 units · 1–2×/week

ROUTE

Subcutaneous injection

FREQUENCY

Daily during loading (8–12 weeks) · 1–2×/week for maintenance · Best taken in the evening to sleep through nausea

CYCLE

8–12 weeks loading → ongoing maintenance as needed

NOTES

UV exposure (even 10–20 min) dramatically amplifies results — MT2 primes melanocytes but UV provides the pigment trigger. 2 mL reconstitution is preferred for cleaner unit math. Protect reconstituted vials from light — store wrapped or in a dark box.

COMMON SIDE EFFECTS

Nausea (80–90% of users during loading — dose in the evening), facial flushing, increased libido, appetite suppression, temporary darkening of existing moles/spots (usually reversible). Do not exceed 2 mg/day.

For informational/educational purposes only. Not medical advice. Not FDA-approved for human use. Consult a licensed physician before use. Research-use only compound.

GHK-Cu

Copper tripeptide · Skin repair · Anti-aging · Daily injection or topical

PURPOSE

Skin rejuvenation, collagen stimulation, wound healing, and cellular repair. Considered the gold standard anti-aging peptide for skin — with documented effects on gene expression related to collagen, elastin, and antioxidant defense.

HOW IT WORKS

Naturally occurring copper-binding tripeptide (Gly-His-Lys). Delivers bioavailable copper to cells, activating pathways for collagen and elastin synthesis, angiogenesis, and anti-inflammatory gene regulation. Levels in the body decline ~60% between ages 20 and 60.

RECONSTITUTION GUIDE · Add bacteriostatic water slowly down the inside vial wall — never directly onto powder. Swirl gently. Never shake.

VIAL SIZE	BAC WATER	CONCENTRATION
50 mg	2.0 mL BAC water	25 mg/mL
50 mg	2.5 mL BAC water	20 mg/mL
100 mg	5.0 mL BAC water	20 mg/mL

DOSE REFERENCE · Based on the most commonly cited research protocol. Assumes standard U-100 insulin syringe (1 mL = 100 units).

DOSE	AMOUNT	U-100 SYRINGE UNITS
Phase 1 (Days 1–15)	1 mg/day	4 units (at 25 mg/mL)
Phase 2 (Days 16–30)	2 mg/day	8 units (at 25 mg/mL)
3×/week alt. protocol	2 mg/dose	8 units (at 25 mg/mL)
Range	1–2 mg/day	4–8 units (at 25 mg/mL)

ROUTE

Subcutaneous injection

FREQUENCY

Daily SubQ injection or 3×/week · Morning preferred · Rotate abdomen sites

CYCLE

30 days on / 30 days off (standard) · 8–12 weeks on / 4 weeks off (intensive)

NOTES

Reconstituted solution should appear clear with a slight blue tint from copper — this is normal. Never freeze reconstituted vials. Some users experience injection site pain (ranging from mild to significant) — if severe, this compound may not be well-tolerated. In that case, topical formulation is a valid alternative: mix reconstituted solution into a thick moisturizer base.

COMMON SIDE EFFECTS

Injection site pain/redness (varies widely by individual), mild bruising. Systemic side effects are uncommon at standard doses. Copper toxicity at research doses is considered extremely unlikely.

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BPC-157

Body Protection Compound · Recovery · Injury repair · Gut health · Daily injection

PURPOSE

Accelerated healing of tendons, ligaments, joints, and muscle tissue. Also used for gut lining repair and as an anti-inflammatory compound. Popular among athletes for maintaining training consistency despite injury.

HOW IT WORKS

A 15-amino acid peptide derived from a protective protein found in human gastric juice. Promotes angiogenesis (new blood vessel formation), upregulates growth hormone receptors in tendon cells, and has documented anti-inflammatory effects. Unusually stable in gastric acid — oral use is also researched for gut applications.

RECONSTITUTION GUIDE · Add bacteriostatic water slowly down the inside vial wall — never directly onto powder. Swirl gently. Never shake.

VIAL SIZE	BAC WATER	CONCENTRATION
5 mg	1.0 mL BAC water	5 mg/mL (5,000 mcg/mL)
5 mg	2.0 mL BAC water	2.5 mg/mL (2,500 mcg/mL)
10 mg	2.0 mL BAC water	5 mg/mL (5,000 mcg/mL)

DOSE REFERENCE · Based on the most commonly cited research protocol. Assumes standard U-100 insulin syringe (1 mL = 100 units).

DOSE	AMOUNT	U-100 SYRINGE UNITS
Conservative	250 mcg/day	5 units (at 5 mg/mL)
Standard	500 mcg/day	10 units (at 5 mg/mL)
Acute injury	500 mcg 2×/day	10 units × 2 (at 5 mg/mL)
With TB-500 blend	300–500 mcg	6–10 units (at 5 mg/mL)

ROUTE

Subcutaneous injection

FREQUENCY

Once or twice daily · Inject near injury site when possible · Rotate sites

CYCLE

8–12 weeks on · 4–6 weeks off · Oral capsule use also documented for gut protocols

NOTES

Often stacked with TB-500 (the 'Wolverine Stack') for synergistic tissue repair — BPC-157 excels at local healing while TB-500 has more systemic reach. Pre-blended 10 mg vials (5 mg each) are widely available and simplify the stack. No lethal dose identified in preclinical research. Banned by WADA for competitive athletes.

COMMON SIDE EFFECTS

Generally very well tolerated. Reported side effects are rare and typically mild: dizziness, nausea, or headache at higher doses. No organ toxicity identified in animal studies.

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TB-500 (Thymosin Beta-4)

Systemic tissue repair · Inflammation · Recovery · Twice weekly loading

PURPOSE

Systemic (whole-body) tissue repair, anti-inflammatory response, flexibility improvement, and recovery acceleration. Works at a broader systemic level compared to BPC-157, making it particularly effective for chronic or widespread inflammation.

HOW IT WORKS

Synthetic version of Thymosin Beta-4, a naturally occurring protein that regulates actin — a protein critical to cell structure and movement. Promotes cell migration to injury sites, reduces inflammatory cytokines, and supports new blood vessel formation across tissues.

RECONSTITUTION GUIDE · Add bacteriostatic water slowly down the inside vial wall — never directly onto powder. Swirl gently. Never shake.

VIAL SIZE	BAC WATER	CONCENTRATION
5 mg	2.0 mL BAC water	2.5 mg/mL (2,500 mcg/mL)
10 mg	2.0 mL BAC water	5 mg/mL (5,000 mcg/mL)
10 mg	4.0 mL BAC water	2.5 mg/mL (2,500 mcg/mL)

DOSE REFERENCE · Based on the most commonly cited research protocol. Assumes standard U-100 insulin syringe (1 mL = 100 units).

DOSE	AMOUNT	U-100 SYRINGE UNITS
Loading phase	2.0–2.5 mg	80–100 units (at 2.5 mg/mL)
Loading phase	2.0–2.5 mg	40–50 units (at 5 mg/mL)
Maintenance	1.25 mg/week	50 units (at 2.5 mg/mL)
With BPC-157 blend	500–1000 mcg	10–20 units (at 5 mg/mL blend)

ROUTE

Subcutaneous injection

FREQUENCY

Loading: 2× per week for 4–6 weeks · Maintenance: 1× per week thereafter

CYCLE

8–12 weeks on / 4–6 weeks off · Often run alongside BPC-157

NOTES

Unlike BPC-157, TB-500 works systemically — inject into the abdomen or thigh rather than near specific injury sites. The standard BPC-157 + TB-500 ratio is approximately 1:4 to 1:5 (e.g., 500 mcg BPC-157 to 2,000–2,500 mcg TB-500). Pre-blended vials typically offer a 1:1 ratio — if using these, adjust total dose accordingly. Banned by WADA.

COMMON SIDE EFFECTS

Generally well tolerated. Reported: fatigue or lethargy immediately post-injection (often resolves within hours), mild headache, injection site reactions. No significant organ toxicity identified in research.

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SEMAX

Nootropic · BDNF enhancement · Cognitive performance ·
Nasal spray or injection

PURPOSE

Cognitive enhancement, neuroprotection, focus, memory, and mental clarity. Approved and used clinically in Russia for stroke recovery and cognitive disorders. Popular in the West as a nootropic compound.

HOW IT WORKS

Synthetic analog of ACTH(4-10), a fragment of adrenocorticotrophic hormone. Stimulates BDNF (brain-derived neurotrophic factor) production — the primary growth factor for neuronal health and plasticity — without the broader hormonal effects of ACTH itself.

RECONSTITUTION GUIDE · Add bacteriostatic water slowly down the inside vial wall — never directly onto powder. Swirl gently. Never shake.

VIAL SIZE	BAC WATER	CONCENTRATION
5 mg	5.0 mL BAC water	1.0 mg/mL (1,000 mcg/mL) — nasal
10 mg	3.0 mL BAC water	3.33 mg/mL (3,333 mcg/mL) — SubQ
10 mg	5.0 mL BAC water	2.0 mg/mL (2,000 mcg/mL) — flexible

DOSE REFERENCE · Based on the most commonly cited research protocol. Assumes standard U-100 insulin syringe (1 mL = 100 units).

DOSE	AMOUNT	U-100 SYRINGE UNITS
Nasal (per spray/drop)	100 mcg	1 pump at 1 mg/mL
Nasal daily dose	200–600 mcg	2–6 pumps at 1 mg/mL
SubQ start (Wks 1–2)	500 mcg/day	5 units (at 3.33 mg/mL)
SubQ target (Wks 3–8)	800 mcg/day	8 units (at 3.33 mg/mL)

ROUTE

Intranasal spray (most common) or subcutaneous injection

FREQUENCY

Once daily in the morning · 5 days on / 2 days off preferred · Avoid late-day dosing

CYCLE

8 weeks on / 4–8 weeks off · Russian clinical protocols use cyclical administration

NOTES

Intranasal is the most-studied and convenient route. Reconstitute 5 mg in 5 mL for 1 mg/mL — each pump delivers ~100 mcg. US regulatory status is evolving (removed from FDA 503A Category 2 list April 2026); verify current sourcing status.

COMMON SIDE EFFECTS

Very well tolerated. Mild nasal irritation (prolonged use), mild headache, temporary fatigue. Avoid other intranasal compounds same session. Do not dose in the evening — interferes with sleep.

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Where to Go From Here

No guide changes anything on its own. What changes things is the decision to act — and then the discipline to keep acting. Start with one section. Make one change this week. Build from there.

The men who make the most progress are not the ones who try to overhaul everything at once. They are the ones who make consistent, small improvements and let them compound over time.

If you found value in this guide and want to go deeper — whether that's specific product recommendations, detailed workout and nutrition programming, style consulting resources, or additional information on the compounds covered in the Peptide Reference Chapter — visit the link below for curated resources, updates, and additional material.

FREE COMPANION RESOURCE

Facial Structure & Exercise Visual Reference

Step-by-step diagrams for every technique in Chapter 08 — mewing, hunter eyes, masseter training, neck work, and nasal breathing. Hosted at the link below.

[YOUR WEBSITE / LINK HERE]

This guide is for informational purposes only and does not constitute medical, health, legal, or financial advice. The peptide reference chapter presents educational information based on publicly available research protocols and is not a prescription, personal recommendation, or treatment plan. Compounds discussed may not be approved by the FDA or other regulatory bodies. Always consult a qualified licensed physician before beginning any peptide, hormonal, or supplement protocol.